SOAP Note

Patient: Patient | **Clinician:** Dr. Yuki Tanaka

Date: Sep 28, 2026 · 10:59 PM | **Encounter ID:** visit-97e282eb

Subjective

Patient reports elevated blood pressure readings of approximately 160/95 mmHg on two separate occasions at home. He has a history of slightly high blood pressure during previous checkups but has never been formally diagnosed with hypertension. He experiences mild headaches but denies chest pain or breathing difficulties.

Objective

Not documented

Assessment

Elevated blood pressure with history of borderline readings; no prior diagnosis of hypertension.

Plan

Confirm with ambulatory BP monitoring; initiate ACE-inhibitor (e.g., Lisinopril 10mg) if confirmed. Advise lifestyle modifications (diet, exercise).

Differential Diagnosis Suggestions

Supportive suggestions only; requires clinician verification and approval.

1. Essential Hypertension (Stage 1) (81%)

Rationale: Persistent elevated BP readings at home (160/95 mmHg) with a history of borderline readings, meeting Stage 1 hypertension criteria per NICE NG136.

2. White-Coat Hypertension (67%)

Rationale: Elevated BP readings at home but no prior diagnosis; could represent white-coat hypertension, though home readings are typically lower in this condition.

3. Tension-Type Headache (64%)

Rationale: Mild headaches could be tension-type, possibly exacerbated by elevated BP, though no clear causal link is established.

4. Secondary Hypertension (Renal) (61%)

Rationale: New-onset elevated BP in a young patient warrants consideration of secondary causes (e.g., renal artery stenosis), though no red flags are present.

5. Caffeine-Withdrawal Headache (50%)

Rationale: Mild headaches could be due to caffeine withdrawal, though no history of caffeine use is documented.

Clinician signature: _____ **Date:** 2026-09-28